

# Detecting Sub-Millimetre Bone Fractures via the X-ray Scatter-to-Primary Ratio

A Monte Carlo (OpenGATE 10 / Geant4) Feasibility Study

[Author Name] • Baanwit Academy • 2026 • preliminary — CL-3 & CL-6 pending

## 1. Introduction

- Hairline / sub-millimetre fractures are frequently **invisible on plain radiographs**: the attenuation contrast they create is tiny.
- Hypothesis**: a fracture perturbs the local scatter field, leaving a signature in the **Scatter-to-Primary Ratio**

$$SPR(x, y) = \frac{I_{\text{scatter}}}{I_{\text{primary}}}$$

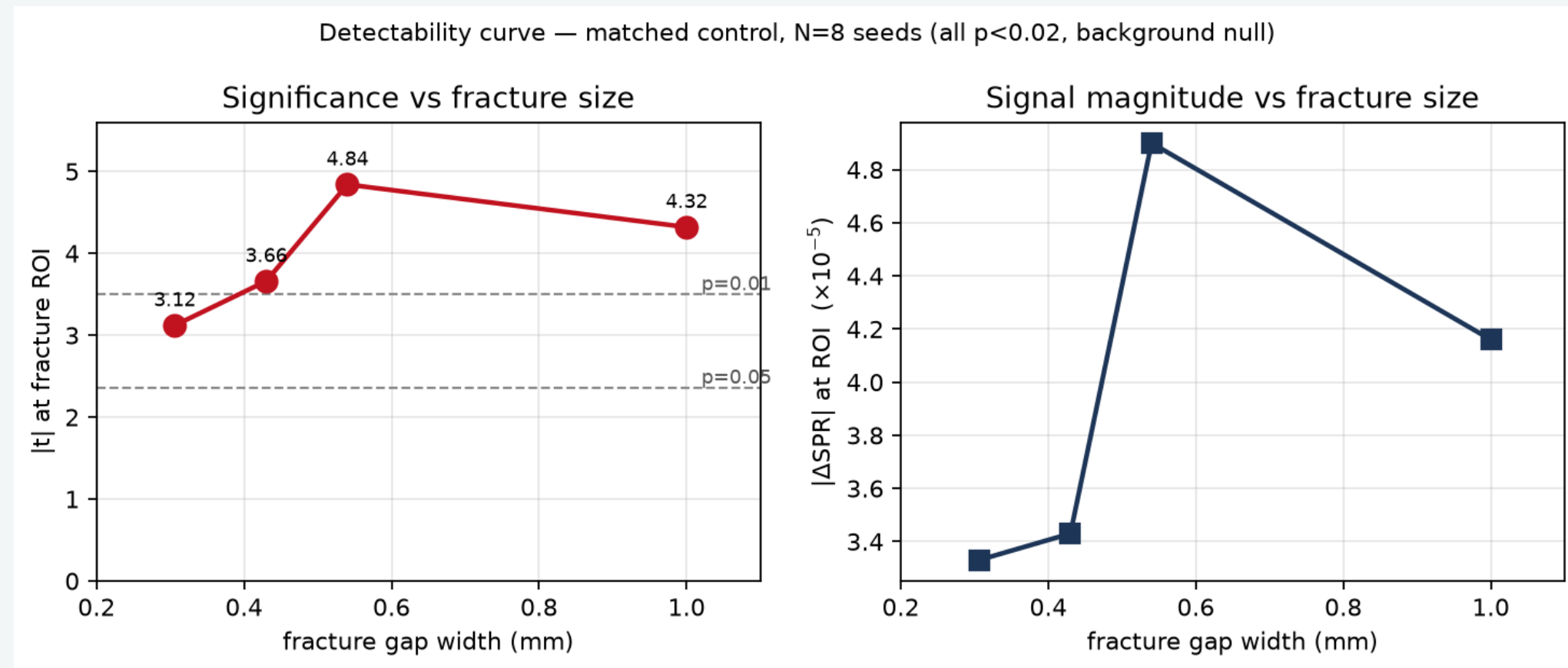
- Aim**: test by Monte Carlo whether this SPR signature is *detectable, statistically robust, not an artifact*, and *physically grounded*.

## 2. Methods

- Engine**: OpenGATE 10 (Geant4), Livermore physics; 80 keV; compact bone (ICRU); AP pelvis; SPR map from primary/scatter separation at the detector.
- Fracture models**: gap widths 0.306, 0.43, 0.54, 1.0 mm.
- Matched control (key)**: the *same* mesh with the gap filled  $\Rightarrow \Delta SPR = SPR_{\text{frac}} - SPR_{\text{ctrl}}$  isolates **only the fracture** (removes the mesh-tessellation confound).
- Multi-seed significance**:  $N=8$  independent seeds, common random numbers; *a-priori* ROI  $t$ -test at the fracture vs. a background ROI.
- Collimation** (40 mm field) concentrates photon statistics on the fracture.

## 3. Detection (CL-0)

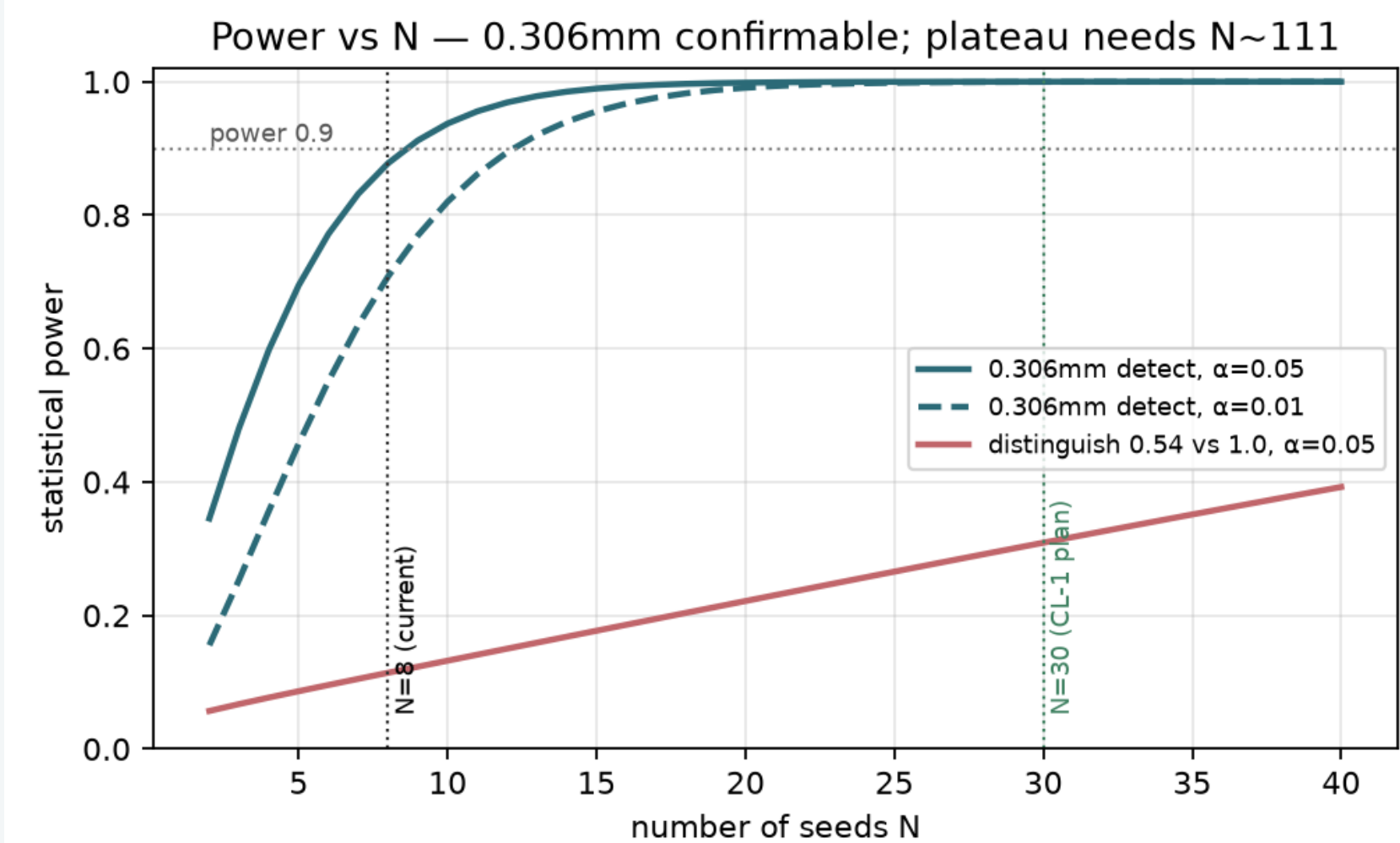
All four sizes detected ( $p < 0.02$ );  $\Delta SPR < 0$  (air gap  $\rightarrow$  more primary  $\rightarrow$  lower SPR); background ROI **null**  $\Rightarrow$  the signal is localised to the fracture.



Signal rises with gap width from 0.306 to 0.54 mm;  $|t| = 3.1, 3.7, 4.8, 4.3$ .

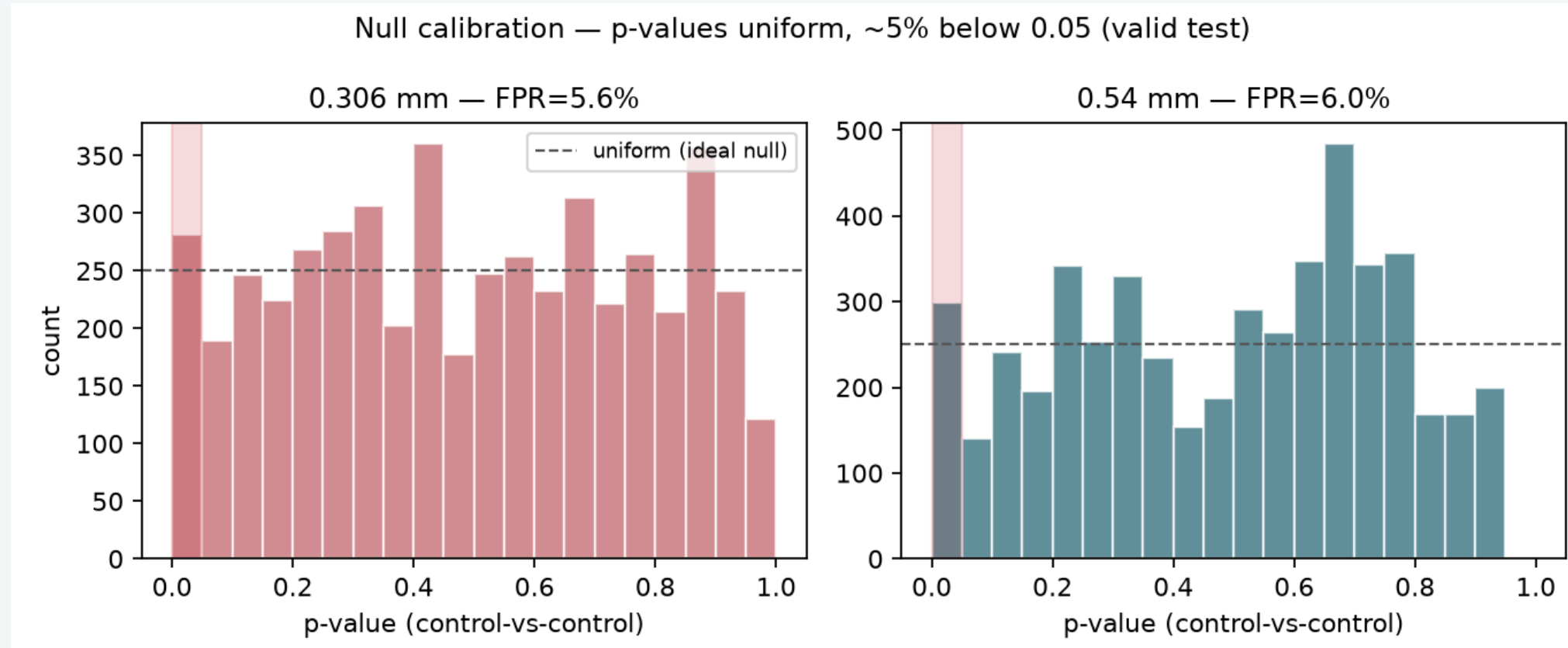
## 4. Robustness & Power (CL-1)

- Bootstrap 95% CIs **exclude 0** for all sizes  $\Rightarrow$  detections are real.
- Smallest fracture (0.306 mm) is **borderline** (80% reliability at  $N=8$ );  $\sim 13$  seeds reach  $p < 0.01$ .



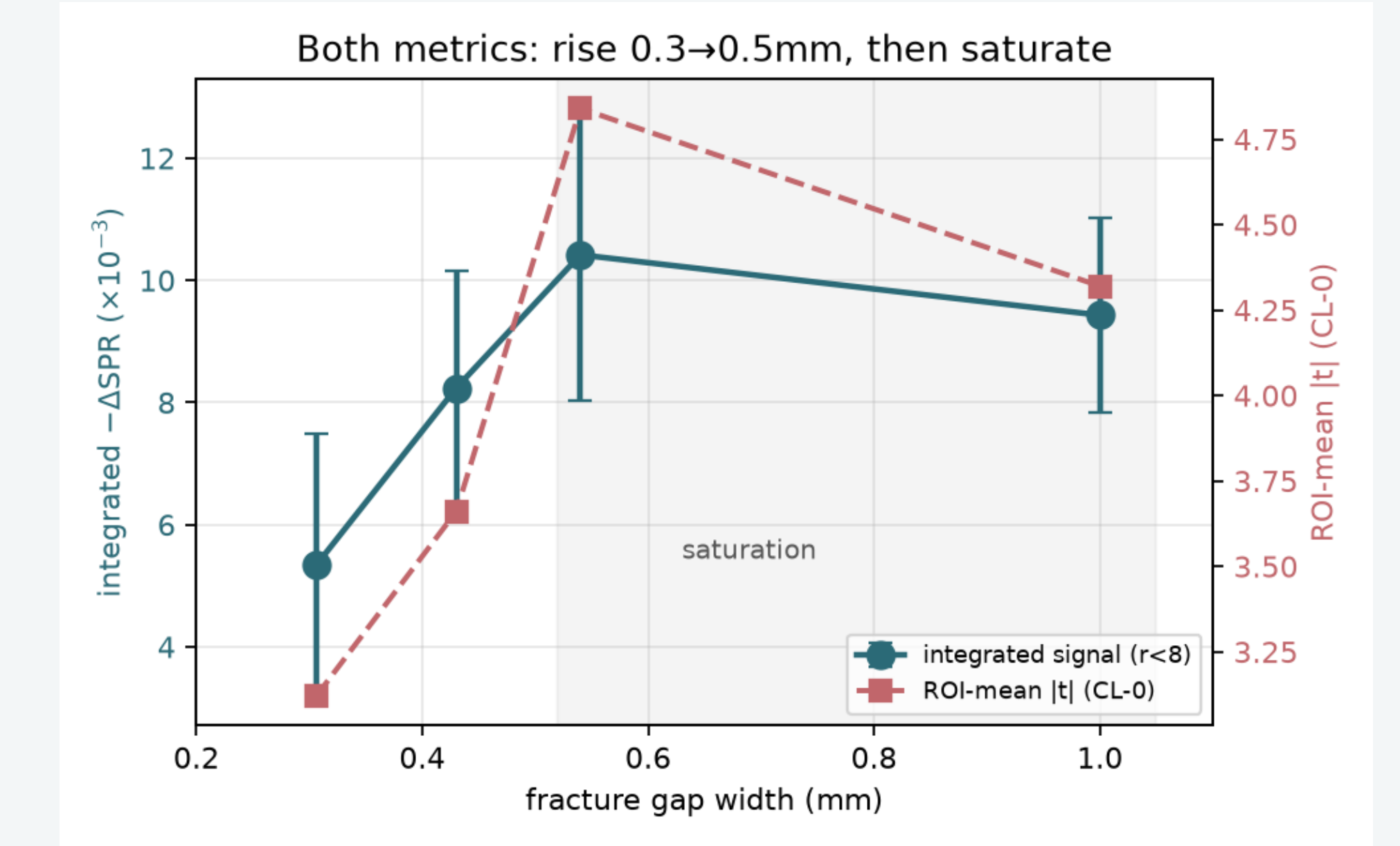
## 5. Validity — no false positives (CL-2)

- Control-vs-control false-positive rate = 5.6–5.8%  $\approx$  nominal 5%.
- Sign-flip permutation confirms significance non-parametrically.
- Only  $\sim 5\%$  of in-beam pixels significant (chance)  $\Rightarrow$  **no global artifact**.



## 6. Plateau resolution (CL-7)

- Signal rises 0.306  $\rightarrow$  0.54 mm then **saturates** ( $0.54 \approx 1.0$ ,  $p=0.73$ ).
- Integrated** (spatial) signal saturates too  $\Rightarrow$  saturation is **physical**, not an ROI artifact.



## 7. Mechanism tests — in progress

**CL-3 Energy dependence** (40–120 keV): does the signal follow photoelectric/Compton physics? **[running]**

**CL-6 Beam-angle dependence**: is the signal maximal when the beam is tangential to the fracture plane (the clinical hallmark)? **[running]**

Results to be inserted in the final version; per-angle geometry pre-computed and queued.

## 8. Conclusions

- The SPR carries a **real, validated** fracture signature detectable to  $\sim 0.3$  mm.
- The signal is **physically bounded**: excellent for *detection*, saturating for *width*  $> 0.5$  mm.
- Energy- & angle-dependence (pending) will complete the mechanistic proof for **Phase 1**.

## 9. Phase-1 evidence pillars

| Detection | Robustness | Validity | Phys. bound | Energy  | Geometry |
|-----------|------------|----------|-------------|---------|----------|
| CL-0      | CL-1       | CL-2     | CL-7        | CL-3    | CL-6     |
| ✓         | ✓          | ✓        | ✓           | running | running  |